



U.S. ARMY



ARMY MEDICINE STRATEGIC PLAN



COMBAT READY CARE ... THIS WE'LL DEFEND!



Awards ceremony for the Expert Field Medical Badge and Expert Soldier Badge, recognizing Soldiers who successfully completed tasks earning them the right to wear these badges. The Expert Field Medical Badge (EFMB) is a prestigious award bestowed upon U.S. Army medical personnel who demonstrate exceptional competence and professionalism in field medical operations. (U.S. Army National Guard photo by 2nd Lt. Timothy Yao)

FOREWORD

Army Medicine Leaders,

The Army Medical Regiment's purpose is clear and enduring, supported by our unifying culture. Collectively, Army Medicine provides combat ready care at the point of need to build trust, inspire confidence, and enable the Army and Joint Force. This sacred mission requires ownership and an understanding that medical leaders at echelon are accountable and responsible for delivering expert medical care and advisement throughout the Army. This establishes trust, and that trust is the cornerstone of everything we do.

Army Medicine is committed to Soldier health, readiness, and lethality across the conflict continuum. This begins at home station by ensuring Soldiers are medically ready for warfighting as the most lethal version of themselves (Soldier as a weapon system). Throughout competition, this lethality is sustained through prevention, health promotion, and force health protection. If deterrence fails, Army Medicine provides combat casualty care at the point of need, clearing the battlefield, and returning Soldiers to the fight. Should Soldiers suffer battle injuries, disease, or illness, Army Medicine is with them every step of the way through rehabilitation. Army Medicine ensures force health protection and sustains the Warfighter for multidomain operations, delivering a medical force that is combat ready in support of Combatant Commanders. We are optimized to support warfighting, providing quality healthcare, and leveraging the whole of government to fight and win our Nation's wars.

A comprehensive strategic plan achieves this end by synchronizing ways and means across commands and components. The Army Medicine Strategic Plan's five Lines of Effort are nested with the Army's Continuous Transformation Campaign Plan: 1) Recruit, Build, and Strengthen the Army Medicine Profession, 2) Transform Army Medicine for MDO and the Future Fight, 3) Sustain Health, Medically Preserve and Reconstitute Combat Power, 4) Deliver Combat Ready Medical Forces, and 5) Strengthen Alliances and Partnerships. This plan is centered on people, our strategic advantage. It embodies our commitment to innovation, leveraging technology and data-driven decision-making. It develops agile leaders and fosters a culture of excellence. It prioritizes collaboration with the joint community while investing in robust partnerships with our allies and partners. At its core, this plan is driven by our unwavering dedication to the health, readiness, lethality, and success of our Soldiers and the Army.

This plan is a dynamic roadmap. Army Medicine will adapt through continuous transformation as our environment and collective knowledge evolve. We expect each of you to embrace and understand the plan's objectives and how they impact your respective organizations. We empower you to contribute to its implementation. Thank you for your commitment to our shared culture, our united purpose, and our sacred mission. We are honored to serve alongside each of you.

Combat Ready Care ... This We'll Defend!



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U.S. Army combat medics with 81st Stryker Brigade Combat Team, Washington National Guard, perform trauma assessments, administer tactical combat casualty care, and evacuate patients during a joint training scenario on Joint Base Lewis-McChord, Washington. (U.S. Army National Guard photo by Staff Sgt. Adeline Witherspoon)

I. INTRODUCTION

The Army Medicine Strategic Plan provides a guiding framework for the U.S. Army's Medical Regiment. The transition of certain legacy Army Medical Command (MEDCOM) organizations to other Army Commands (ACOMs) and the Defense Health Agency (DHA) renewed the integration role of The Surgeon General. The Surgeon General (TSG) executes vital statutory responsibilities under Title 10, as well as integrating health service support functions across the joint, combined, interagency, and Army force. These responsibilities include optimizing Army Medicine to support the mission, the Soldier, and their Families. TSG accomplishes this by executing medical aspects of manning, training, and equipping the Total Army Medicine Force, advising the Secretary and Chief of staff of the Army on health service support and force health protection, and as integrator, synchronizes Army medical capabilities.

Army Medicine is grounded in providing combat-ready care optimized in support of the mission, the Soldier, and the Family. Firmly aligned with the Army Chief of Staff's areas of focus, this prioritizes warfighting as the primary purpose of the Army and its *raison d'être*. Since the formation of our Nation, the Army's enduring motto, "This We'll Defend," has reflected its unwavering commitment to defend the Nation. To fulfill this purpose, the Army concentrates on four focus areas:

Warfighting: Developing the capabilities to dominate on the battlefield

Delivering Ready Combat Formations: Ensuring that units are prepared and equipped for deployment

Continuous Transformation: Embracing innovation and adaptation to stay ahead of emerging threats

Strengthening the Profession: Fostering a culture of excellence, teamwork, and leadership among all personnel

By aligning with these key areas, Army Medicine will be optimized for support to the Chief of Staff's priorities through integrating with our Army, the Joint Force, and our Allies and Partners. This will ensure that every Army medical professional and medical formation is trained, ready, and integrated into a cohesive medical force.

II. STRATEGIC ENVIRONMENT

The National Defense Strategy prioritizes four key lines of effort: defending the U.S. homeland and its hemisphere, deterring China through strength rather than force, increasing burden sharing between the U.S. and its allies and partners, and supercharging America's defense industrial base. To achieve this, the Department of War (DoW) will focus on southern border security and countering regional narco-terrorism. Simultaneously, the DoW will deliver a ready, globally deployable force to deter China and will deepen partnerships with "model allies" who demonstrate a commitment to collective defense and invest in their own military capabilities. Underpinning these efforts, the Department will lead an aggressive modernization of systems, secure supply chains, and accelerate fielding advanced technologies to ensure a lethal, effective force.



U.S. Soldiers assigned to the Regimental Support Squadron, 2d Cavalry Regiment, transport a simulated casualty from the medical evacuation area to the triage area for rapid injury assessment and prioritized treatment on Grafenwoehr Training Area, Bavaria, Germany. (U.S. Army photo by Spc. Elizabeth Krause)

To support this, Army Medicine is investing in a transformation mindset and an agile culture to meet emerging and future threats. Fighting near-peer adversaries, the intensity and duration of Large Scale Combat Operations (LSCO), lack of air superiority, extended lines of communication, contested and congested logistics, combined with the use of advanced weaponry, create an environment with increased risk of illness, injury or death. The physical and mental demands of sustained combat operations will take a toll on Soldiers, increasing risk of fatigue, decreased situational awareness, and accidents or mistakes. As a result, LSCO is projected to result in a higher number of casualties, including killed in action (KIA), wounded in action (WIA), disease and non-battle injuries (DNBI). To mitigate these risks Army Medicine will prioritize force health protection (FHP), develop effective tactics, techniques, and procedures (TTPs) for LSCO, leverage the Joint Trauma System, improve casualty care at the point of need, enhance mass casualty operations, prepare for prolonged care, and prioritize rehabilitation of casualties in theater.

The character of medicine is also changing, and Army Medicine will transform to provide care to Soldiers, Families, and Retirees, both domestically and abroad, across the continuum of competition to conflict. This transformation will address gaps in our ability to execute evacuation, prolonged care, contested logistics, FHP, health service support (HSS), and command and control functions in LSCO. Army Medicine will strengthen the Army Medical Department (AMEDD) by addressing critical factors such as retention, professional development, and recruiting challenges.

The Army Medicine landscape has shifted from the legacy structure to a more integrated and collaborative model. As an integrator, Army Medicine has developed strong relationships with key stakeholders, including Transformation and Training Command, Army Materiel Command, the Army Service Component Commands, sister services, DHA, the Joint Staff, and partners and allies to leverage innovation, inform doctrine, and ensure integrated logistics and operational planning.



Lt. Gen. Mary K. Izaguirre, U.S. Army Surgeon General, tours a combat casualty tent staffed by U.S. Army combat medics and medical officers with the 1st Armored Brigade Combat Team during the medical experimentation portion of Project Convergence Capstone 4 (PC-C4), Fort Irwin, California. Team members with the U.S. Army Medical Materiel Development Activity are assessing the progress of two programs during PC-C4: the Health Readiness and Performance System (HRAPS) and Medical Casualty Predictive Logistics Utilization System (MCPLUS). (U.S. Army National Guard photo by T. T. Parish)

III. ASSUMPTIONS

Constrained Resources: The military continues to face resource pressure and must find ways to operate effectively and efficiently to achieve objectives. We must address the challenges of recruiting and retaining medical personnel. Both face increased pressure due to competition from civilian healthcare organizations, job satisfaction and service identity. The military relies on a combination of recruitment, retention, and training strategies to man medical billets while utilizing incentives, bonuses, and educational opportunities to attract and retain top talent.

Increasing Complexity and Uncertainty: The global security environment continues to be complex and uncertain, with a wide range of threats emerging across multiple domains. Alliances and partnerships will become increasingly important for the U.S., and building and maintaining these relationships will be critical to achieving national security objectives.

Continued Advancements in Technology: Technological advancements will drive changes in the character of warfare. The U.S. must stay ahead of the curve in developing and fielding new technologies while leveraging Artificial Intelligence and Machine Learning to facilitate care and decisions at the speed of war. The future of warfare will be characterized by high casualty rates in LSCO, and the U.S. must be prepared to operate effectively across all domains (land, maritime, air, space, and cyberspace) and dimensions (human, physical, and information) to maintain freedom of maneuver in light of increased casualties.

Transformation: Army Medicine will fully integrate with established Army processes and ensure TSG, as the principal medical advisor to the Secretary of the Army and the Chief of Staff of the Army, has visibility of and input to the health and medical aspects of manning, training, and equipping the Army. Specifically, TSG provides technical advice and assistance to the Secretariat and Army Staff (ARSTAF) for matters regarding public health, readiness of the force, Soldier recovery care, medical force structure and equipping. TSG is also the primary advisor for medical force development, materiel, research and development, training and education, evacuation, and military construction.



A Blackhawk takes off with a simulated casualty during a medical evacuation rehearsal ahead of Joint Pacific Multinational Readiness Center 25-02, at Donnelly Training Area, near Fort Greely, Alaska. The Large Scale Combat Operations scenario used in JPMRC 25-02 helps 11th Airborne Division Soldiers and leaders develop and refine the tactics, techniques, and procedures necessary to successfully operate in remote and extreme Arctic winter conditions and overcome both environmental and military challenges. (U.S. Army photo by Pfc. Makenna Tilton)

IV. CHALLENGES AND OPPORTUNITIES

Challenges:

Recruiting and Retaining Medical Personnel: Army Medicine faces challenges in recruiting and retaining medical personnel, namely physicians, nurses, and other healthcare professionals, due to competition from the civilian healthcare market, a national shortage of healthcare professionals, and the changing character of the military medical enterprise.

Mass Casualty Management: The current operational environment poses significant risks in managing mass casualties, including the need for rapid triage, evacuation, and treatment of large numbers of wounded, ill, and injured personnel.

Medical Evacuation: Agile, multimodal medical evacuation will be required to clear the battlefield and maximize treatment capability in LSCO. Rapid evacuation may be complicated by near-peer adversaries, the inability to establish air superiority, distances, terrain, and security concerns.

Chemical, Biological, Radiological, and Nuclear (CBRN) Threats: The unpredictable catastrophic consequences of CBRN threats pose a threat to the force and can create mass casualties, widespread contamination, panic, and disruption of medical infrastructure and safe patient movement.

Health Surveillance and Disease Prevention: Army Medicine maintains situational awareness of operationally significant health threats in garrison and training environments to ensure the medical readiness of Army combat forces. In the deployed environment, Army Medicine must monitor and mitigate risks of endemic, emerging, and re-emerging diseases and other health threats.

Clear the Battlefield, Return Soldiers to Duty, and Overcome Contested Logistics: Army Medicine must provide rapid and effective treatment, evacuation, medical logistics, and

care in austere and urban environments. Army Medicine will leverage innovative solutions, such as telemedicine, expeditionary equipment, casualty evacuation, and enhanced logistics, to deliver rapid and effective care. Army Medicine will also rebuild its intra-theater rehabilitation capabilities so that Soldiers can return to the fight as quickly as possible.

Trauma Care: The nature of wounds sustained in peer conflict differs from those experienced in counterinsurgency operations (COIN). In COIN, wound patterns are characterized by explosive blasts, gunshot, and shrapnel wounds. In contrast, the Ukraine conflict showed a rise in high-velocity wounds from artillery, rocket, and missile strikes. These resulted in complex polytrauma cases with severe blast, thermal, and blunt force wounds, coupled with psychological trauma from constant drone and artillery attacks. Successful treatment of these complex wounds requires a well-defined, capability-based trauma system.

Disease and Non-Battle Injuries (DNBI): DNBI historically accounts for a significantly higher erosion of combat power and increased demand on all roles of care and medical evacuation systems. To mitigate this impact, Army Medicine must enhance theater medical capabilities including operational public health and equipment sets at Role 2 facilities and rehab hospitals.

Multidrug Resistant Bacterial Infections: Army Medicine must develop effective strategies to combat the escalating threat of multidrug resistant bacterial infections. This particularly impacts prolonged care, delayed evacuation, rehabilitation in theater, and potential transmission to the homeland through evacuation to military, Veterans Affairs (VA), and civilian hospitals.

Opportunities:

Partnerships and Collaborations: Army Medicine will expand partnerships and collaboration with academic, civilian healthcare facilities, allies, partners, and other government agencies to leverage new technologies, share best practices, develop cost-effective acquisition strategies, and advance the field of military medicine.

Telemedicine: Telemedicine will play a key role in Army Medicine and Joint Force capabilities in both competition and crisis, enabling virtual consultations, monitoring, and treatment of Soldiers in remote and austere environments.

Medical Decision Advantage Data Strategy: Real-time knowledge of battle injuries and DNBI provides tactical, operational, and strategic advantage over adversaries. Army Medicine's ability to rapidly recognize and take action to mitigate conditions eroding combat power could prove decisive in maximizing the human as a weapon system.

Machine Learning including Artificial Intelligence (AI): Leveraging machine learning aligns with the Army's transformation initiatives enabling Army Medicine to provide more effective, and more efficient care to its patients, both on and off the battlefield.

Build Resilience: The Ukraine conflict has underscored the importance of grit and resiliency in modern warfare. To instill resiliency in our Soldiers, the Army must employ programs that build mental toughness and resilience, enhancing Soldier readiness and performance in high-stress environments.

V. OPERATIONAL APPROACH

The Army Medicine Strategic Plan focuses on five Lines of Effort, with component objectives depicted in Figure 1. As our legacy organizations transferred to ACOMs, our focus has shifted to integration, convergence, advising, and enabling. This strategic plan optimizes Army Medicine's limited resources to enable the Army's success in current and future operational environments.

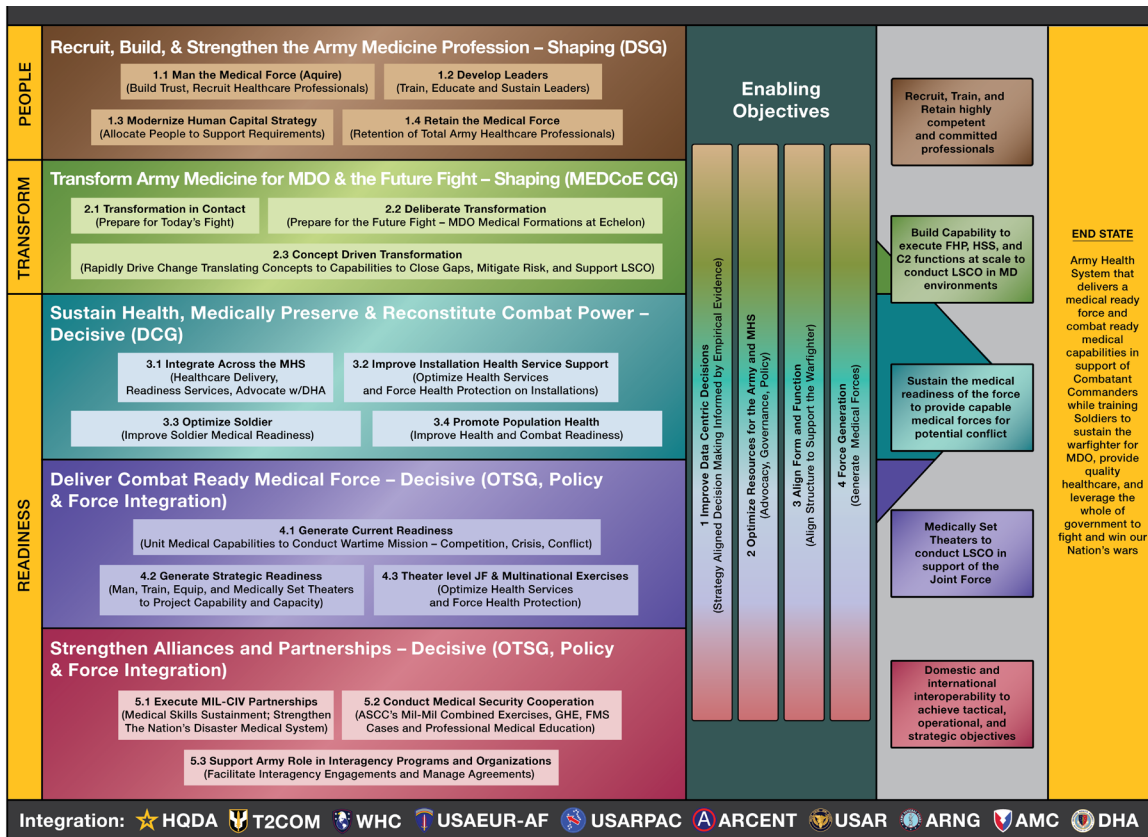


Figure 1. Army Medicine Strategic Plan

LOE 1. Recruit, Build, and Strengthen the Army Medicine Profession (Shaping). Army Medicine implements a 21st century talent management strategy, leveraging data-driven decision making to develop leaders and improve personnel outcomes. This approach integrates stakeholders to foster confident and competent leaders through continuous and deliberate training, education, and assessment. Additionally, Army Medicine will improve its recruiting and retention efforts by optimizing job satisfaction and financial compensation for healthcare professionals, maximizing return on investment, and enhancing quality of life for service members and their Families.

Objective 1.1. Man the Medical Force (Acquire). Army Medicine will mitigate ongoing recruitment and retention challenges to ensure a ready medical force. TSG is integrated with Army human resource enterprise to optimize recruiting, training, retention, and quality of life. Army Medicine will synchronize its efforts with Army initiatives to leverage human capital strategy, data analytics, and legislative reform to attract and retain top medical talent.

Task 1.1.1. Develop and execute a national AMEDD Marketing Campaign to enhance recruiting platforms (i.e. website(s), conventions attendance, and university/college visits).

- Task 1.1.2. Maximize AMEDD student recruitment (i.e. USUHS, MS HPSP, MC|VC|DC HPSP).
- Task 1.1.3. Optimize targeted use of accession pay for critical medical specialties and evaluate ROI.
- Task 1.1.4. Streamline the application process; reduce waiver processing times.
- Task 1.1.5. Maximize retention of COMPO 1 talent exiting service into COMPO 2 and COMPO 3 for continued service.
- Task 1.1.6. Maximize the use of hiring flexibility and streamline hiring times to recruit eligible civilians.

Objective 1.2. Develop Leaders. Army Medicine will develop agile leaders for a dynamic and uncertain future. This development spans the entire career of our officers, warrant officers, non-commissioned officers, enlisted service members, and civilians through a deliberate, continuous, and progressive process. Army Medicine recognizes that a proactive talent management system must support readiness and modernization, fundamentally improving the leadership development, management, and support to the Army's number one priority - People.

- Task 1.2.1. Educate, train and sustain Army Medicine (Profession of Medicine).
- Task 1.2.2. Optimize talent assessment and placement Tools (Profession of Arms).
- Task 1.2.3. Develop strategies to increase CSL opt-In rates.
- Task 1.2.4. Increase Army Medicine publications within the Army's Harding Project.

Objective 1.3. Modernize Human Capital Strategy. Army Medicine will execute deliberate talent management, providing a continuous learning environment that leverages each individual's knowledge, skills, and behaviors to maximize their potential, growth and mission success throughout their career.

- Task 1.3.1. Improve AMEDD distribution process - MEDCOM Allocation of Assets (MA2).
- Task 1.3.2. Engage and retain talented civilians; Use internal process to effectively consider current civilian employees for non-competitive placement into vacancies.
- Task 1.3.3. Improve Individual Mobilization Augmentee (IMA) and Active Guard Reserve (AGR) utilization.
- Task 1.3.4. Expand career portability between COMPOs.

Objective 1.4. Retain the Medical Force. Army Medicine will mitigate ongoing recruitment and retention challenges to provide requisite capacity and capability in support of the Army. TSG will optimize recruiting, training, retention, and quality of life. Army Medicine will synchronize its efforts with Army initiatives to leverage human capital strategy, data analytics, and legislative reform to attract and retain top medical talent.

- Task 1.4.1. Increase Job Satisfaction.
- Task 1.4.2. Leverage station of choice opportunities.
- Task 1.4.3. Maximize Department of the Army Career Engagement Survey (DACES) and other targeted survey tools to gain officer retention motivator; develop predictive modeling analytics.
- Task 1.4.4. Maximize the use of retention special pay rates for critical AOCs.
- Task 1.4.5. Maximize the use of retention incentives to retain eligible civilians.

LOE 2. Transform Army Medicine for MDO and the Future Fight (Shaping). Army Medicine remains effective in and responsive to future conflicts. To achieve this, TSG ensures the Army has the necessary medical capacity and capabilities in competition and crisis to provide premier health service support in competition and conflict. As DoW undergoes healthcare reform, Army Medicine will transform its formations and capabilities to meet the Army's multi-

domain force requirements, ensuring readiness and capability for future fights. Additionally, Army Medicine engages the appropriate Research, Development and Acquisition (RDA) enterprise to advocate for Army requirements and funding for Science and Technology (S&T) and Research, Development, Test and Evaluation (RDT&E) appropriations. As the operating environment evolves, Army Medicine will develop threat-informed capabilities that enable the 10 interrelated Army medical functions. From capability- development to planning and programming medical field systems for the warfighter, medical force modernization stakeholders will deliver battlefield medical capability and capacity. These capabilities will be synchronized across the doctrine, organization, training, materiel, leadership and education, personnel, facilities, and policy (DOTMLPF-P) continuum.

Objective 2.1. Transformation in Contact. Transformation in Contact (TiC), conducted over eighteen to twenty-four months, focuses on prototyping organizational changes and integrating emerging technology. This iterative process – allowing for rapid learning, failure, refinement, and solution development – is critical for maintaining a competitive advantage. Building upon the momentum of TiC, Continuous Transformation accelerates the implementation of objectives outlined in the Army Campaign Plan and informs delivery of the total force envisioned in the forthcoming Army Warfighting Concept.

Task 2.1.1. Identify and nominate medical capabilities for TiC consideration.

Task 2.1.2. Assess prototype formation design impact to medical capability and capacity.

Task 2.1.3. Identify and message transformation activities focused on the AHS imperatives: Clear the Battlefield, Maximize Return to Duty, and Overcome Contested Logistics.

Objective 2.2. Deliberate Transformation. Deliberate transformation uses existing Army processes (Total Army Analysis (TAA) and PPBE) to program and build formations over the next two to seven years.

Task 2.2.1. Incorporate medical capabilities into Army governance processes and develop medical force modernization strategic communication.

Task 2.2.2. Synchronize DOTMLPF-P integration of capabilities across the 10 medical functions and advocate for risk-informed resourcing in the Strategic Portfolio Analysis Review/Program Objective Memorandum.

Task 2.2.3. Identify and submit Force Design Updates (FDUs) necessary to support continuous transformation.

Task 2.2.4. Assess impact of the Army Structure (ARSTRUC) changes annually to the medical force structure.

Objective 2.3. Concept Driven Transformation. Concept driven transformation looks into the future to identify new concepts, formations, talent, doctrine, technology, or other DOTMLPF-P changes required to compete in the future.

Task 2.3.1. Identify necessary capability or threat based assessments derived from the Army Warfighting Concept.

Task 2.3.2. Identify and incorporate medical concept required capabilities consistent with Army Warfighting Concept and link S&T efforts and investments to close gaps.

Task 2.3.3. Identify and synchronize experimentation outcomes with approved gaps for shared understanding within transformation.

Task 2.3.4. Generate Army and adopt Joint Capability Requirement Documents based on DOTMLPF-P recommendations.

LOE 3. Sustain Health and Medically Preserve and Reconstitute Combat Power (Shaping).

The Surgeon General ensures the Army's medical force can provide combat-ready Soldiers. To address future challenges, including increased casualties and emerging threats, Army Medicine must enhance training and advance medical research to sustain the health of our force. Collaboration between key stakeholders supports optimization of Soldier performance and population health through the emerging technologies and expertise. Army Medicine keeps faith with Families, providing world-class health service support, with joint and interagency partners, at home and abroad.

Objective 3.1. Integrate Across the Joint Medical Enterprise. Army Medicine is an essential partner in the joint medical enterprise, integrated to develop and maintain Soldier Medical Readiness, ensure the readiness of our deployable medical forces, and fulfill its responsibilities to man, train, and equip the future medical force.

- Task 3.1.1.** Collaborate With Assistant Secretary of War for Health Affairs (ASW(HA)) and Assistant Secretary of the Army for Manpower and Reserve Affairs (ASA(MR&A)) to ensure DoW health policies and programs address the Army's Operational Medicine (OPMED) requirements.
- Task 3.1.2.** Define Army provider readiness, medical R&D, disease surveillance, and blood support requirements and assess DHAs ability to provide opportunities or deliver support.
- Task 3.1.3.** Lead and advise CCMD/ASCC/ACOM surgeons to identify and meet OPMED requirements.

Objective 3.2. Improve Installation Health Service Support. Installation Health Service Support Plans (IHSSP) synchronize and optimize HSS and FHP in support of the installations, formations, and Army Senior Leaders. The IHSSP's establish measures of effectiveness and measures of performance to inform resource prioritization, allocation, and distribution in support of the Army Mission.

- Task 3.2.1.** Establish IHSSP Common Operating Picture (COP) for MTF Commanders/Directors and capability managers.
- Task 3.2.2.** Assess the performance of Army IHSSP's.
- Task 3.2.3.** Assess Veterinary Services mission execution (Food Protection, Animal Health Care, Veterinary Preventive Medicine).

Objective 3.3. Optimize Soldier Performance (Lethality). The Army is focused on optimizing Soldier performance through the application of knowledge, skills, and emerging technologies to improve resilience, well-being, and health. This encompasses the five domains: physical readiness, nutritional readiness, mental readiness, spiritual readiness, and sleep readiness. TSG incorporates physical performance, nutrition, mental health, cognitive performance, environmental health, and sleep, to optimize the Total Force Fitness through the Army's Holistic Health and Fitness (H2F) program.

- Task 3.3.1.** Build operationally relevant and standardized performance metrics and practices to enable individual, collective, and community readiness.
- Task 3.3.2.** Support Soldiers to self-monitor personal performance capabilities and capacities.
- Task 3.3.3.** Develop strategies to prevent and reduce Soldier performance decrements from oath to Expiration Term of Service (ETS).
- Task 3.3.4.** Maximize Soldier medical readiness through collaboration with key stakeholders.

Objective 3.4. Promote Population Health. Enhancing population health is a strategic advantage that boosts the overall readiness of Army forces by preventing illnesses and injuries, thereby maximizing operational effectiveness. Operational public health support and force health protection activities align with Army and DoD bio-surveillance programs. Key initiatives include anticipating and mitigating health threats, monitoring occupational and environmental health through surveillance and epidemiology, and coordinating emergency responses through public health emergency management and bio-surveillance. These efforts are crucial to ensuring the well-being of the Total Army force and align with the Army Surgeon General's priority of Sustaining Health, which is nested within the Army's strategic area of Delivering Ready Combat Formations.

Task 3.4.1. Support operational public health.

Task 3.4.2. Provide public health emergency management.

Task 3.4.3. Align Force Health Protection activities with Army & DoD bio-surveillance programs.

LOE 4. Deliver Combat Ready Medical Force (Decisive). Army Medicine's primary responsibility is to support warfighter readiness through agile, adaptive, and responsive medical capabilities. To achieve this, Army Medicine will assess and mitigate medical risks, integrate medical capabilities into exercises, and coordinate with partners to ensure a combat ready medical force. TSG plays a key role in integrating medical capabilities to provide medically credible forces and sustain the health of the total force.

Objective 4.1. Generate Current Readiness. Army Medicine facilitates the medical readiness of units and identifies any gaps that need to be addressed before exercises or deployments through operational assessments. Ensures that medical units have the requisite personnel, training, equipment, and supplies, to support generation of medical ready forces for combat.

Task 4.1.1. Develop and provide medical readiness policies, systems, and reporting mechanisms to identify readiness lag measures and positively influence lead measures.

Task 4.1.2. AMEDD generates sufficient current readiness to meet demands for assigned, allocated, and immediate response forces.

Task 4.1.3. Review readiness reporting to assess risks associated with personnel, training, equipment status, and supply levels.

Objective 4.2. Generate Strategic Readiness. Strategic readiness is the Army's ability, through time and space, to provide combatant commanders with trained and ready units that can meet the joint force's operational demands. The Army measures strategic readiness by assessing seven strategic readiness tenets (manning, training, equipping, sustaining, installations, capacities and capabilities), as well as the Joint Capability Area (JCA) assessment for HSS.

Task 4.2.1. Integrate into the DoW and HQDA risk reporting mechanisms (e.g., Army Strategic Readiness Assessment Inputs, Annual Joint Assessment, Joint Medical Estimate).

Task 4.2.2. Assess CLVIII HQDA Centrally Managed Programs (Medical Materiel Readiness Program (MMRP), Unit Deployment Package (UDP), Medical Chemical Defense Materiel (MCDM) Quarterly).

Task 4.2.3. Assess medical field system equipment fill rates across the total force quarterly.

Task 4.2.4. Assess SRC 08 readiness to meet Operation Plan (OPLAN) and Time Phased Force Deployment Data (TPFDD) requirements.

Task 4.2.5. Assess Army Prepositioned Stock (APS) readiness for SRC 08 sets.

Objective 4.3. Theater Level, Joint Force, and Multi-National Exercises. Army Medicine plays a crucial role in ensuring that medical capabilities are effectively employed in theater-level, joint forces, and multinational exercises. This involves a multi-faceted approach to integrate medical support into these complex operations, enhancing interoperability, and ensuring the health and readiness of all participating forces. Army Medicine works closely with joint force commanders and multinational partners to integrate medical planning into overall operational plans.

Task 4.3.1. Assess, identify, and integrate opportunities within the Joint-Exercise Priority List (JEPL) and Army-Exercise Priority List (AEPL).

Task 4.3.2. Consolidate and distribute potential training and exercise opportunities across Army Medicine.

Task 4.3.3. Conduct knowledge management of exercise plans, and lessons learned.

LOE 5. Strengthen Alliances and Partnerships (Shaping). Army Medicine leverages agreements as well as collaboration and coordinating mechanisms with military and civilian partners to meet strategic objectives. By strengthening relationships, Army Medicine builds trust, collaboration, and interoperability that enhances the ability to provide care on the battlefield, respond to emerging health threats, and develop novel medical response capabilities. The interagency and inter-governmental relationships are vital to the DoD's success in assuring casualty care from point of injury through rehabilitation.

Objective 5.1. Execute Military-Civilian Partnerships Programs. Army Medicine creates military-civilian partnerships (MCPs) with civilian trauma centers, pre-hospital EMS teams and other civilian organizations to build and maintain readiness of the medical force. MCPs bridge a gap in the Army's ability to provide high volume /high acuity patient exposures within its medical treatment facilities to sustain clinical proficiency in surgery, trauma, and critical care. Programs with civilian partners build both individual and collective competencies, enhancing integrated approaches and sharing of best practices. A network of MCPs that are engaged in joint training activities enhance mil-civ interoperability which strengthens the National Disaster Medical System.

Task 5.1.1. Develop overarching HQDA MCP policy.

Task 5.1.2. Develop and maintain a central registry of Army MCPs.

Task 5.1.3. Monitor and assess readiness outcomes of Army medical personnel training at MCPs.

Task 5.1.4. Document factors that affect the establishment and/or effective operation of MCPs.

Objective 5.2. Conduct Medical Security Cooperation and Security Assistance. In support of the CCMD theater priorities, the Army builds globally integrated military health service support to assure care for the warfighter from point of injury through rehabilitation. Global Health Engagement (GHE) and Medical Security Cooperation (MSC) Operations, Activities, and Investments support a ready medical force, assure interoperability with allies and partners, and build access and influence with emerging partners in potentially contested environments.

Task 5.2.1. Identify personnel across the Army with a formal responsibility of coordinating and planning MSC/GHE activities.

Task 5.2.2. Develop Army MSC/GHE policy and doctrine.

Task 5.2.3. Develop Army competencies at each echelon for planning and conducting MSC/GHE.

Task 5.2.4. Develop a GHE data governance for medical C2 integration, and lessons learned.



U.S. Navy corpsmen with Bravo Surgical Company, 3rd Medical Battalion, load a simulated casualty into a U.S. Army UH-60Q Black Hawk during joint casualty evacuation (CASEVAC) and hoist training, Marine Corps Base Hawaii. Corpsmen and flight medics collaborated to hoist simulated casualties onto a UH-60Q Black Hawk medical evacuation helicopter to increase joint interoperability and practice CASEVAC procedures. (U.S. Marine Corps photo by Lance Cpl. Hunter Jones)

Objective 5.3. Support Army role in Interagency programs and organizations. Army Medicine maintains relationships, develops interagency agreements, participates in interagency governance, and represents Army leadership in Interagency decision forums to facilitate the training, certification, and operational employment of Army and Army Medicine personnel. The National Disaster Medical System is a core element of the Integrated CONUS Medical Operations Plan design and process. This plan requires multiple Interagency and DoD organizations to conduct large scale patient movement in CONUS. Coordination of this objective occurs through a network of liaisons and program managers to comply with public law and execute missions assigned in DoD policy.

- Task. 5.3.1.** Develop Army NDMS and Army Medical Defense Support of Civil Authorities policy.
- Task. 5.3.2.** Formalize Army roles in Medical DSCA Capabilities Based Assessments, Joint publications, Army policies, and EXORDs.
- Task. 5.3.3.** Liaise and develop DoD and Interagency plans and policies. Integrate Army roles and responsibilities into Interagency plans, policies and incident annexes that support Emergency Support Function 8 public health and medical response for natural disasters and national security incidents.
- Task. 5.3.4.** Develop best practices and lessons learned from medical DSCA operations. Using DoD and Army forums, Centers of Excellence (COE) and academic institutions promulgate best practices to improve combat casualty care and clinical skill performance.

Enabling Objectives (Shaping): The enabling objectives crosscut the five Lines of Effort. By leveraging data as a strategic asset, Army Medicine effectively integrates, influences, and drives innovation. These data-driven insights enhance our ability to steward resources in support of the mission, the Soldier, and their Families. To maximize mission success, Army Medicine aligns structure and personnel to optimize form and function to achieve desired effects.

Enabling Objective 1. Make Data-Based Decisions. Successful execution of this objective enables Army Medicine to support the Army's data-centric transformation; leveraging data as a strategic asset to enhance combat readiness, anticipate strategic and operational requirements, and sustain the health of the force. Accurate and timely medical information underpins the decision advantage that Army Medicine brings through Army Health System support. Health information must enable commanders to clear the battlefield of casualties, return wounded, injured, and ill Soldiers to the fight, overcome contested logistics, and protect Soldiers against health threats. Timely access to this information enables commanders at echelon to make informed decisions regarding the employment and engagement of the force. TSG's role in mission accomplishment is to sustain the health of the Total Force and man, train, and equip the medical force. Decision analytics enable that role.

Task 1.1. Cultivate a data-centric culture.

Task 1.2. Design the system.

Task 1.3. Develop decision advantage.

Task 1.4. Employ enterprise advantage.

Task 1.5. Leverage relationships to achieve success.

Enabling Objective 2. Optimize Resources for the Army. Army Medicine professionals effectively manage funding through prioritizing, aligning, validating, and documenting requirements. Optimizing resources enables Army Medicine to be agile, responsive, and relevant in support of the Army. They secure resources by validating requirements in each phase of the PPBE process. They monitor, prioritize, and manage budget execution in support of Army priorities.

Task 2.1. Develop and sustain Army Medicine human resource common operating picture.

Task 2.2. Align and advocate for Army Medicine priority requirements (Authorizations/FTEs/Funding).

Task 2.3. Document Army Medicine program requirements, capabilities, capacity and risk.

Task 2.4. Effectively maximize and manage MEDCOM funding; monitor and influence medical equities for the Army and Army DHP readiness funding.

Enabling Objective 3. Align Form and Function. Army Medicine will align form and function to support the Army and warfighters in a resource constrained environment.

Task 3.1. Codify TSG Integrator Role.

Task 3.2. Align structure to mission directive.

Task 3.3. Fully implement theater medical commands.

Task 3.4. Validate requirements through model/study.

Enabling Objective 4. Force Generation. The Surgeon General executes Title 10 training authorities through the Graduate Medical Education (GME)/ Graduate Dental Education (GDE)/ Phase 2 (PH2) Clinical Training Programs/ Long Term Health Education and Training (LTHET) / MEDCoE and Medical Education and Training Campus programs to generate Combat Ready Medical Forces. This enables the Army to meet the demands of combatant commanders, while also ensuring units are properly trained, equipped, and manned to execute assigned missions. The force generation tasks are incorporated in the five Lines of Effort.

Implementation and Assessment: The Army Medicine Strategic Plan is a multi-faceted approach nested in the Army's Continuous Transformation Campaign Plan, focusing on five Lines of Effort: Recruit, Build, and Strengthen the Army Medicine Profession, Transform Army Medicine for MDO and the Future Fight, Sustain Health and Medically Preserve and Reconstitute Combat Power, Deliver Combat Ready Medical Forces, and Strengthen Alliances and Partnerships. To accomplish our multiple objectives, we will track both the achievement of outcomes and task execution. We will measure the success of LOEs and their objectives through Measures of Effectiveness (MOEs) and Measures of Performance (MOPs), including leadership development metrics, medical readiness rates, and alliance and partnership engagements. MOPs track our efforts, while MOEs assess if our efforts achieve desired impact. An assessment battle rhythm of accountability enables the Surgeon General to identify where integration, influence, and innovation are most needed. By regularly tracking and evaluating, we will assess plan effectiveness, identify areas of excellence or improvement, and make data-driven policy, process, and resource allocation decisions to inform future strategy.

VI. CONCLUSION

The Army Medicine Strategic Plan provides a comprehensive framework for Army Medicine to support the Army's warfighting mission and ensure the health and readiness of its Soldiers. Through the implementation of this plan, Army Medicine will prioritize force protection, develop and implement effective tactics and procedures, foster a culture of excellence and innovation, ultimately ensuring that Army Medicine is trained and ready in support of Army's Mission, its Soldiers, and their Families.

U.S. Soldiers carry a simulated casualty on a litter while responding to a mass casualty situation during Installation Protection Exercise '24 at Tower Barracks, Grafenwoehr, Germany. The exercise involved a simulated vehicle rollover that resulted in seven trauma casualties, who were treated and stabilized by Army ambulances, medical evacuation teams, and host-nation emergency medical services. Medical personnel performed triage, administered life-saving treatments, and transferred the casualties via ground and air evacuation to host-nation hospitals. The exercise tested the clinic's ability to execute critical tasks in a high-stress environment and validated the readiness of Army medical personnel to respond to real-world emergencies. (U.S. Army photo by Sgt. Gianna Elle Sulger)





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